Critical Discourse on Hospice Palliative Care Outreach

Azeez Abidemi Adigun

Brock University

ADED 3P41 Community Outreach and Adult Literacy

Karen Minty

October 9, 2026

Critical Discourse on Hospice Palliative Care Outreach

Ty Mabee's presentation examined the Hospice Palliative Care Outreach program offered by Haldimand-Norfolk Community Senior Support Services. The program provides free and confidential support to people with a life-limiting illness, their families, and caregivers. Services include care planning, caregiver support, volunteer respite, grief groups, and safeTALK suicide alertness training. The presentation clearly explained that hospice outreach works alongside clinical care. This matters because serious illness affects more than medical needs. It can change family roles, cause emotional stress, and require difficult decisions about care and loss.

The presentation connected the program to community development through place, interaction, change, and social capital. Haldimand and Norfolk cover a large rural area, so place affects access. Interaction appears in relationships among clients, families, volunteers, health professionals, and care coordinators. Change occurs as adults adjust to illness, caregiving, and loss. Social capital is visible in the time, knowledge, and trust that volunteers and community partners provide. This relationship-based approach is a strength because people facing serious illness may be more willing to accept support from trusted local organizations.

The program also reflects adult education principles. Volunteer training provides knowledge adults can apply in real situations. Care planning and consent support self-direction because the person receiving care must agree to the referral and participate in decisions. Grief groups allow adults to reflect and learn through others' experiences. SafeTALK is problem-centred because participants practise noticing suicide risk and responding. These activities value experience, relevance, participation, and immediate application (Mukhalalati & Taylor, 2019). However, public information does not show whether learning outcomes are assessed. Attendance alone cannot prove that participants gained confidence or changed how they respond.

Critical Questions and Potential Issues

The most important question raised by the presentation is whether the program reaches residents equally across the region. Haldimand and Norfolk had a combined population of 116,706 spread across approximately 2,848 square kilometres in the 2021 Census (Statistics Canada, 2022). A referral process can be open in principle while remaining difficult to use in practice. People may still face long travel distances, limited transportation, weak internet access, delayed responses, or low awareness of the service. It would be useful to know the average wait time, the number of unmet requests, and whether participation differs by municipality, age, income, language, disability, or access to transportation. Without this information, it is difficult to determine whether the program's benefits are distributed fairly.

A second question concerns community participation. The program relies on volunteers and local relationships, but it is unclear how much clients, caregivers, and bereaved family members influence its design. Are people with lived experience asked what they need and what prevents

participation? Community development should involve residents in decisions rather than treat them only as service recipients. A client and caregiver advisory group could review outreach, training, accessibility, and service gaps. The organization should also avoid depending too heavily on unpaid volunteers, who need clear roles, supervision, emotional support, and manageable workloads.

A third issue is how the program measures success. Free services, several referral routes, trained volunteers, and long-standing regional work are meaningful assets, but they do not show the effect of support. The program could examine whether clients feel respected, caregivers feel less isolated, volunteers feel prepared, and grief-group participants find the sessions useful. Short surveys, interviews, and follow-up conversations could provide numerical results and personal feedback. The organization should also explain how it protects confidentiality when collecting sensitive information.

Alternatives and Community Value

Several additions could strengthen access without changing the program's relationship-based purpose. The organization could offer rotating outreach sessions in smaller communities, partner with libraries or community centres for private meeting space, and provide telephone or virtual options when appropriate. A transportation partnership could support people who cannot travel. The program could also train local community ambassadors to explain available services and referral steps. These approaches should be tested with residents because a solution that works in one rural community may not work in another.

Overall, the program has clear value because it addresses needs that clinical treatment alone cannot meet. It supports care decisions, caregivers, volunteers, and people experiencing loss. The presentation showed strong social capital and adult learning, while leaving questions about reach, equity, participation, and outcomes. The program would be stronger if it reported these results and involved residents in deciding how services should grow.

References

Mabee, T. (2026). Hospice palliative care outreach: Haldimand-Norfolk Community Senior Support Services [Digital presentation]. ADED 3P41 Community Outreach and Adult Literacy, Brock University.

Mukhalalati, B. A., & Taylor, A. (2019). Adult learning theories in context: A quick guide for healthcare professional educators. Journal of Medical Education and Curricular Development, 6, Article 2382120519840332. https://doi.org/10.1177/2382120519840332

Statistics Canada. (2022, February 9). Population and dwelling counts: Canada, provinces and territories, census divisions, census subdivisions (municipalities) and designated places https://doi.org/10.25318/9810000401-eng